Captopril

FORM Oral Solution 25mg/5ml

INDICATION Hypertension
Congestive Cardiac Failure

DOSE RANGE

N.B. The neonatal response to treatment with ACE inhibitors is very variable, and some neonates develop profound hypotension with even small doses; a test- dose should be used initially and increased cautiously

AGE	DOSE	FREQUENCY	ROUTE
<37 weeks gestational age	10 microgram/kg	**Single Test Dose**	Oral
	If tolerated, give 10 to 50 microgram/kg/dose and increase as tolerated/required by 50-100microgram/kg/dose Usual max dose 300micrograms/kg/day in divided doses Doses up to 500micrograms/kg/day can be used*	2 - 3 times daily	Oral
≥ 37 weeks gestational age	10-50 microgram/kg	**Single Test Dose**	Oral
	If tolerated, give 10-50microgram/kg/dose and increase as tolerated/required in increments of 100micrograms/kg/dose to Max dose 2mg/kg/day in divided doses	2 - 3 times daily	
1month – 6 months (**and** >37 weeks CGA)	100 microgram/kg	**Single Test Dose**	Oral
	If tolerated, give 100 – 300 microgram/kg/dose and increase as tolerated/ required to max. 4mg/kg **daily in divided doses**	2 - 3 times daily	Oral

Neonatal

* Higher doses of up to 1mg/kg/dose have been used in neonates in conditions such as dilated cardiomyopathy with intractable heart failure - doses exceeding 500microgram/kg/dose should not be used without Consultant approval. Discussion of such cases with a Cardiologist is advised.

CAUTIONS, CONTRA-INDICATIONS AND SIDE EFFECTS
- See Summary of Product Characteristics and most recent edition of BNF for Children

STORAGE

Once opened, the solution is stable for 21 days at room temperature.

FURTHER INFORMATION

- For doses less than 500micrograms the 5mg/ml oral solution should be diluted as follows

Captopril 5mg/ml oral solution	0.4ml
Water for Injection	Up to 20ml

Resulting Solution 2mg in 20ml (100micrograms/ml)

SPECIAL MONITORING

- Continuous cardiac monitoring may be required
- Monitor blood pressure every 15 minutes for 1 to 2 hours during test dose.
- Monitor U&E's prior to and during treatment
- Monitor WBC every 2 weeks for first 3 months of treatment (risk of neutropenia)
- Monitor for proteinuria particularly in patients with renal impairment

LICENSED STATUS

Licensed for all ages

APPLICABLE POLICIES

West of Scotland Neonatal Guidelines:

Consult local policy if applicable

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Dose may vary depending on indication, age, renal function, hepatic function, and concomitant medications.
This monograph should be used in conjunction with the terms of reference document prepared by the West of Scotland Pharmacist Group. Information is correct at the time of publication and as per local practice agreement.
For further advice contact your clinical pharmacist or pharmacy department.